

Beyond Survival: Addressing the Hidden Crisis of Post-Stroke Living Conditions in Hong Kong's Elderly

Wong Sze Chun (Word count: 1884)

Abstract

Hong Kong's healthcare system has achieved high stroke survival rates in recent years, yet this success masks a failure in addressing the profound challenges of survivorship, leading to a diminished quality of life for post-stroke individuals. Drawing on data from my community home visits experience, I hope to raise attention to the hidden crises of environmentally unsuitable housing, psychological distress, and social isolation of post-stroke patients. It further points out the limitations of current rehabilitative services, which are clinically focused and time-constrained. This essay proposes a shift towards integrated, community-based care, focusing on a mode where primary healthcare, social services, and volunteer networks converge to create a social support network, ensuring survival with improved quality of life for Hong Kong's stroke survivors.

Introduction

According to the latest data recorded by the Centre for Health Protection (CHP, 2025), the number of deaths from cerebrovascular diseases has decreased from approximately 3,500 to 3,000 annually in recent years. However, this medical success story often concludes at the hospital door. For thousands of elderly survivors like Mr. Chan, discharge marks the beginning of a hidden battle—one not against necrosis of brain tissue, but against the loss of independence, dignity, and purpose. (HKU STROKE, n.d.). Mr. Chan, in his seventies, a stroke survivor with a drooping left eyelid and weakened left-side mobility, exemplifies this systemic shortcoming. His case, encountered during a visit as a student volunteer with the Generations Connect Project through the Aberdeen Kai Fong Welfare Association (AKA), serves as an anchor for this discussion and the initiative for me to share my viewpoint. Based on our interview, I believe that the current post-stroke care in Hong Kong has been overly focused on clinical survival, overlooking the basic living environment and psychosocial recovery. Using Mr. Chan's narrative, this essay will hopefully cover most of the gaps between survival and survivorship in their overall post-stroke experience.

Case Report

Mr. Chan's situation illustrates how an unsuitable living environment can become a prison of hidden dangers. His public housing flat, a fortress of obstacles, is characterised by poor ventilation and a musty odour. When advised to open a window, his response was "Yes, I can open it, but it is really hard for me to close it back". It is a consistent risk assessment, the trade-off between fresh air and personal safety. This shows us that post-stroke individuals may lose control over their environment. It significantly impacted their quality of life, self-worth, and confidence. This suggests a pressing need for practical assistive technologies and environmental modifications to empower them in their survivorship.

Furthermore, Mr. Chan's social world has drastically diminished. His reliance on a walking frame underscores a high fall risk, a leading cause of morbidity and mortality in post-stroke elderly (Olivera D., 2021). Consequently, his life is confined to his home and a nearby NGO centre. When asked about socialising, he consistently attributes his reclusiveness to being "lazy". This was actually Mr. Chan's excuse to refrain from a long journey. We learnt that this laziness is due to his left-sided hemiparesis. He feels exhausted after just a few steps, and he needs to use his walking frame for every step he takes. Using a defensive mechanism, which is known as self-handicapping in psychology, whereby individuals adopt a pretext for anticipated failure to protect their self-esteem. (Howard, S.F., 2023). He wore the label 'lazy', preferring to blame his personality rather than admit his helplessness. It was a heartbreaking lie, a way to hide the problems he faced in his basic life. In Mr. Chan's case, the label of "laziness" masks the debilitating effects of post-stroke fatigue, which is a common symptom; around 25%-85% of stroke survivors experienced it and learned helplessness evolved from repeated failure. (Y. Dong, 2024). This makes him socially deprived and lonely all the time. His retreat into endless short videos on the China platform on his phone is not enjoyment but an escape from reality. This indicates social deprivation and loneliness that current Hong Kong healthcare systems are ill-equipped to address.

This psychosocial decline is compounded by a loss of personal pleasure and unresolved guilt. Mr Chan ironically said that he went to the bar before nearly every day with friends to enjoy food and drinks, such that in the current days, he can only eat bland meals provided by the centre and stay at home for the whole day.' His complaints about the "too healthy," bland meals provided by the centre are a strong stance for missing pleasure. This highlights a gap in care, which is the neglect of culinary joy and mental well-being in the pursuit of secondary prevention. While nutritional management is important, a lack of culinary joy can exacerbate post-stroke depression. There is a need for more all-rounded dietary support that balances health in such needy. Moreover, his regretful narrative of "debt" for a past life of "binge drinking" and

“unlimitedly eating heavily oiled food” also reflects to me that there is a need for mental health services for post-stroke individuals. To improve the recovery of post-stroke depression, post-stroke counselling should address self-forgiveness and adaptation rather than blame for beforehand uncontrolled hypertension or vascular-related diseases.

Gaps Identified

Mr. Chan is not an outlier in current Hong Kong society. His story is the story of thousands, revealing systemic failures in the healthcare structure. As suggested by several public healthcare talks, the failure is rooted in a fundamentally inverted care model in HK. An effective health system is conceptualised as a pyramid, with robust primary and community care forming the broad base of support, reducing the need for acute hospitalisation and intensive care. However, as noted in public health forums, Hong Kong’s model is paradoxically top-heavy. A disproportionate share of funding and resources is allocated to hospital-based treatment. Through observations from over 80 home visits, I have identified consistent and critical gaps.

Environmental Mismatch:

Hong Kong’s ageing housing increases, particularly in public estates, are ill-equipped to support a population with physical disabilities, especially those who experienced hemiparesis after stroke. According to research, 65% of stroke victims have experienced hemiparesis (Sophie, W., 2016), showing the urgency of housing improvement support. Recurrent observations across more than thirty public estates in various territories reveal common hazards: narrow doorways that cannot accommodate walking frames, bathrooms devoid of essential grab bars, and high door thresholds that may cause tripping hazards. Evidence has shown that home modifications can increase patient independence and reduce the burden on caregivers (Phillippa, C., 2019). While home improvement services exist to perform home modification, as in Mr. Chan’s case, he has received grab bars in his bathroom. Yet, the scope and funding of the home improvement team remain insufficient.

Psychosocial Abyss:

Beyond the physical environment, a profound psychosocial mood disorder also affects recovery. Post-stroke depression (PSD) is a prevalent yet commonly neglected phenomenon, affecting an estimated 20-60% of survivors; however, it remains grossly underdiagnosed and under-treated in community settings (Dmitry, F., 2022). The loss of

social roles, coupled with isolation like that of Mr. Chan (whose sole relative, a sister, is absent due to work), creates a devastating cycle where mental health decline impedes motivation and engagement in physical rehabilitation. (Robert, G., 2016). The healthcare system's focus on biomedical outcomes often leaves mental well-being unaddressed.

Rehabilitation Desert:

A critical barrier to functional recovery is the existing rehabilitation desert, where necessary services are theoretically available but practically inaccessible. A lot of the elderly tell us that subsidised physiotherapy will need to queue for a period until they get an appointment, and the frequency is insufficient for full recovery. It may cause the loss of the critical early window for neuroplasticity and recovery, a period identified as most cost-effective within the 5-14 days post-stroke using rats as a clinical model. (Platz, T., 2021)

Furthermore, standard PT/OT is often confined to clinical settings. As Mr. Chan's case illustrates, this model fails to address the specific, practical challenges of a patient's unique home environment, such as navigating furniture layouts or manipulating stiff window latches, tasks essential for regaining independence.

Besides that, Mr. Chan complains to us that the trip to the hospital requires 1 hour if taking the bus downstairs. He is so frustrated with travelling to just get rehab or a follow-up appointment.

This combination of temporal, contextual, and geographical inaccessibility creates a rehabilitative desert, leaving patients unsupported in the healthcare system.

Solutions and Discussions

To improve the quality of life of post-stroke individuals, we must move beyond hospital-centric care. We propose a community care framework to deliver sustained and tailored support directly into their home environments. This proactive approach ensures that recovery is not an isolated clinical exercise but a holistic process of reintegration into daily life.

Firstly, by the extensive deployment of specialised community occupational therapists for environmental audits. Their role would be to identify hazards (loose rugs, poor lighting) and prescribe simple, low-cost adaptations (lever-style door handles, grab bars, window opening aids). This turns the home from an obstacle course into a

therapeutic environment. It helps systematically transform the home from an obstacle fortress into a safe and enabling therapeutic environment that actively facilitates post-stroke patients to live in.

Secondly, developing elderly centres like AKA into mental health support centres. This includes training staff to screen for PSD. These elderly centres are already trusted community touchpoints, the role can be expanded from passive service providers, where you provide services when being asked, to active mental health facilitators, actively calling or visiting isolated elderly every 2 days. These hubs can directly combat the social isolation and stigma that fuel the psychosocial challenges faced by post-stroke patients.

Nutritional support can also be improved by transforming from bland, medically compliant meals to empowering patients with culinary autonomy. This can be achieved by designing healthy and adaptive meals with professional food nutritionists. This approach respects the critical need for secondary prevention while actively restoring joy and a sense of control associated with eating, addressing the lament of patients like Mr. Chan who miss the gustatory pleasure of food.

Finally, the strategic adoption of technology enabled care coordination. Tele-rehabilitation platform providing patients like Mr. Chan who have access to their phone with immediate access to guided exercise videos prescribed by their physiotherapist. Hence, instead of using the phone to watch short videos to solve boredom, this can allow individuals to begin rehabilitation as soon as possible, even before or in between physiotherapy appointments.

Conclusion

Mr. Chan's unopened window is more than a household detail; it is a barrier between a life saved and a life lived. Hong Kong has mastered the science of helping people survive strokes; it now must embrace the art of helping them live beyond them. This requires a fundamental reimagining of caregiving—from a medical model that ends at discharge to a social, community-based model that begins there. By integrating clinical expertise with community resources and human connection, we can build a system where no survivor is ever called "lazy" for a struggle they did not choose. We can replace obstacles with adaptations and isolation with integration. The goal is not just providing a meal, but enabling a person to cook for themselves. Not just to add years to life, but to add life to years, ensuring that for elderly stroke survivors in Hong Kong, survival is the starting point, not the end goal.

References

- Centre for Health Protection. (2025). *Number of registered deaths by leading cause of death, 2001-2024*. Retrieved August 25, 2025, from <https://www.chp.gov.hk/en/data/4/10/27/380.html>
- HKU Stroke. (n.d.). Stroke discharge procedure. *The University of Hong Kong*. Retrieved August 27, 2025, from <https://www.stroke.med.hku.hk/caregivers-support/stroke-discharge-procedure>
- Djurovic, O., Mihaljevic, O., Radovanovic, S., Kostic, S., Vukicevic, M., Brkic, B. G., Stankovic, S., Radulovic, D., Vukomanovic, I. S., & Radevic, S. R. (2021). Risk Factors Related to Falling in Patients after Stroke. *Iranian Journal of Public Health*, 50(9), 1832–1841. <https://doi.org/10.18502/ijph.v50i9.7056>
- Friedman, H. S. (Ed.). (2023). *Encyclopedia of mental health* (Third edition.). Academic Press.
- Yuan, D., Salwismawati, B., Salziyan, B., & Linxi, T. (2024). Post-stroke fatigue interventions for stroke survivors: A scoping review. *Belitung Nursing Journal*, 10(6), 601–613. <https://doi.org/10.33546/bnj.3526>
- Wist, S., Clivaz, J., & Sattelmayer, M. (2016). Muscle strengthening for hemiparesis after stroke: A meta-analysis. *Annals of Physical and Rehabilitation Medicine*, 59(2), 114–124. <https://doi.org/10.1016/j.rehab.2016.02.001>
- Carnemolla, P., & Bridge, C. (2019). Housing Design and Community Care: How Home Modifications Reduce Care Needs of Older People and People with Disability. *International Journal of Environmental Research and Public Health*, 16(11), 1951. <https://doi.org/10.3390/ijerph16111951>
- Frank, D., Gruenbaum, B. F., Zlotnik, A., Semyonov, M., Frenkel, A., & Boyko, M. (2022). Pathophysiology and Current Drug Treatments for Post-Stroke Depression: A Review. *International Journal of Molecular Sciences*, 23(23), 15114. <https://doi.org/10.3390/ijms232315114>
- Robinson, R. G., & Jorge, R. E. (2016). Post-Stroke Depression: A Review. *The American Journal of Psychiatry*, 173(3), 221–231. <https://doi.org/10.1176/appi.ajp.2015.15030363>
- Platz, T. (2021). *Clinical Pathways in Stroke Rehabilitation: Evidence-based Clinical Practice Recommendations* (1st ed. 2021.). Springer Nature. <https://doi.org/10.1007/978-3-030-58505-1>